GREATER NEW YORK HOSPITAL ASSOCIATION

PRESIDENT, KENNETH E. RASKE • 555 WEST 57TH STREET, NEW YORK, NY 10019 • T (212) 246-7100 • F (212) 262-6350 • WWW.GNYHA.ORG

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Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
National Coordinator for Health Information Technology
Department of Health and Human Services

Subject: HHS Health Sector AI RFI [RIN0955-AA13]

Dear Dr. Keane:

On behalf of the more than 200 hospitals and health systems across New York, New Jersey, Connecticut, Pennsylvania, and Rhode Island that make up the acute care membership of the Greater New York Hospital Association (GNYHA), thank you for the opportunity to comment on the Request for Information (RFI) on Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care [RIN0955-AA13].

As the Department of Health and Human Services (HHS) continues to establish its role in artificial intelligence (AI) policy, specifically with respect to research, reimbursement, and regulation, we believe it is important to keep in mind the existing regulatory framework that currently exists with respect to hospital operations. Hospitals operate under regulations that specify privacy, security, and safety standards that capture the use of technology, including AI. For example, the Medicare Conditions of Participation (CoPs), 42 C.F.R. Part 482, are standards of quality and safety that hospitals must comply with to receive Medicare and Medicaid reimbursement.[1] CoP standards cover patient rights, quality assessment and performance improvement, compliance with existing laws, and a governing body that is responsible for the oversight of the medical staff, patient care, and safety. The Health Insurance Portability and Accountability Act of 1996 (HIPAA) Privacy and Security rules dictate national standards with respect to the protection of protected health information, in addition to data access and security.[2] Additionally, the US Food and Drug Administration's (FDA) regulation of Software as Medical Devices includes AI diagnostics and digital therapeutics, requiring adherence to quality systems, cybersecurity, and clinical evaluation.

Rather than creating duplicative or redundant regulations for hospitals, HHS should evaluate whether standards for Medicare participation, HIPAA, or FDA guidance are needed for private (nonhospital) vendors. This approach would support innovation and avoid creating excessive regulatory burden. Supporting hospitals with a thoughtful regulatory approach and appropriate reimbursement pathway will facilitate AI adoption in health care.

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[1] 42 C.F.R. Part 482. Conditions of Participation for Hospitals. https://www.ecfr.gov/current/title-42/chapter-IV/subchapter-G/part-482
[2] Health Insurance Portability and Accountability Act of 1996, Pub. L. No. 104-191, 110 Stat. 1936 (1996) https://www.govinfo.gov/link/plaw/104/public/191

Here are GNYHA's responses to the questions included within the RFI.

1. ***What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?***

Innovation in AI for health care may be stalled by the following barriers.

For hospitals, there is limited real-world evidence regarding clinical effectiveness and return on investment. Hospitals typically adopt new technologies based on demonstrated improvements in patient outcomes, operational efficiency, or cost savings. Hospitals are therefore balancing investing in new tools with managing the risk of unknown return on investment (ROI). In addition, hospitals that do not have a robust IT infrastructure do not have a straightforward mechanism for evaluating private external AI vendors. In GNYHA's discussions with its safety net hospital members, hospital representatives have indicated that hospital electronic health record (EHR) vendors have created AI tools, and they are generally comfortable with using those EHR-based tools. In contrast, it takes significant effort to evaluate other private vendors to ensure patient safety and security standards are met.

Clinicians and ancillary staff have also expressed concerns about AI inconsistencies or hallucinations, implicit biases, and unintended consequences such as additional administrative burdens. Clinicians are also especially concerned about keeping the "human in the loop" to ensure clinical decision making is not left solely to an AI tool. Patients and community members have similarly raised concerns about data privacy, biases, and the potential for AI to exacerbate existing disparities.

Lastly, **there is no clear or consistent reimbursement pathway for many AI-enabled clinical services or infrastructure support for hospitals integrating such tools.** The rapid introduction of AI-enabled clinical services necessitates developing a payment strategy that recognizes the infrastructure, oversight, and workforce investments required to implement these tools safely and effectively, especially for safety net and rural hospitals that may not have the initial infrastructure to implement such tools.[3]

2. ***What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.***

Regulation

HHS is uniquely positioned to align regulation, in addition to reimbursement and research, among its regulatory agencies to support effective AI integration in clinical care. Coordinated activity, along with

[3] Harvard Medical School Center for Health Policy. Transforming health care with AI: Effective reimbursement can lead to better care and lower costs. Harvard Medical School Center for Health Policy website. June 11, 2024. https://hcp.hms.harvard.edu/news/transforming-healthcare-ai-effective-reimbursement-can-lead-better-care-and-lower-costs

a national AI policy, would promote broader adoption and reduce confusion regarding applicable guidance and requirements.

HHS should establish a regulatory framework for clinical AI use across HHS, where priorities and requirements are aligned. Harmonizing expectations across the FDA, Centers for Medicare & Medicaid Services (CMS), Assistant Secretary for Technology Policy/The Office of the National Coordinator for Health Information Technology, and Office for Civil Rights would provide more clarity for hospitals and health systems as well as the private vendors with which they work. GNYHA supports the intent expressed in Executive Order 14365, Ensuring a National Policy Framework for Artificial Intelligence, that there should be a unified national standard.[4] **While an overarching Federal regulatory framework is important, we urge HHS to leave flexibility for state-specific AI requirements where appropriate.** For example, as the regulators of insurance, states are rapidly introducing AI-related laws related to limiting the ability of health insurance companies to use AI to deny medical claims and require prior authorization without a human in the loop. Allowing some flexibility will allow each state to adopt policies that are responsive to priorities expressed by health care organizations regarding payer abuses.

Reimbursement

Federal reimbursement policy has historically played an important role in the adoption of new health care technologies. For example, the incentive payments in the Health Information Technology for Economic and Clinical Health (HITECH) Act of 2009 led to a rapid and measurable increase in hospital EHR adoption nationwide.[5] Similarly, CMS's new technology add-on payments (NTAP) have driven moderate adoption of newly developed technologies in inpatient settings.[6] Hospitals adopt and scale technologies when reimbursement structures recognize and support the associated costs.

However, hospitals are at different stages of investing in AI-enabled tools. While some have begun using AI functionalities across their entire health system, others are unable to consider developing or purchasing basic tools due to significant capital constraints and tight margins. HHS should be cautious in developing payment policy that further widens the "digital divide," where rural and other underserved areas have poorer access to AI and digital services. AI adoption requires substantial upfront capital, workflow redesign, training, governance oversight, cybersecurity protections, and ongoing monitoring.

To best support the adoption of valuable new AI tools, **HHS through CMS should first and foremost focus on ensuring adequate reimbursement through the payers that fall within its purview.** Medicare and Medicaid continue to reimburse hospitals at rates below the cost of providing care. In its

[4] 90 FR 58499. Executive Order 14365. Ensuring a National Policy Framework for Artificial Intelligence. December 11, 2025.
[5] Gold, M., & McLaughlin, C. Assessing HITECH Implementation and Lessons: 5 Years Later. *The Milbank Quarterly.* 94(3), 2016; 654–687. https://doi.org/10.1111/1468-0009.12214
[6] Tsuruo, S., Schlacter, J., Dhruva, S. S., Ross, J. S., & Horwitz, L. I. Services and payments associated with the Medicare new technology add-on payment program. *Health Affairs Scholar*. 2025; 3(1). https://doi.org/10.1093/haschl/qxae182

March 2025 report to Congress, the Medicare Payment Advisory Commission (MedPAC) estimated an aggregate Medicare operating margin in 2023 of -12.6% and projected a similar margin for 2025. MedPAC continues to estimate negative Medicare aggregate margins even for hospitals it identified as being relatively efficient (i.e., performing well in the domains of cost and quality).[7]

In recent years, MedPAC has also raised concerns about Medicare payment increases being insufficient to protect beneficiary access. In its March 2023 report, MedPAC noted that underestimates of the growth in hospital input prices for several years post-pandemic have contributed to payment inadequacy and recommended Medicare payment updates above current law for both fiscal year (FY) 2024 and FY 2025. Notably, this was the first time it recommended an increase over the current law update for hospital services. These recommendations were not adopted by Congress.

CMS should focus on providing market basket updates for the Inpatient Prospective Payment System (IPPS) and Outpatient Prospective Payment System (OPPS), and payment updates for the Physician Fee Schedule (PFS) that correct for past updates and better reflect the cost of providing care to patients.

CMS will likely need to handle reimbursement for AI tools differently depending on the payment system. IPPS, with its inpatient episode-based reimbursement, could be a reasonable pathway to reimburse for AI tools used in the inpatient setting in conjunction with CMS improving payment adequacy. Notably, payment rates will not reflect the resource use associated with AI-enabled tools for some time, as those rates are based on old data. Therefore, CMS should consider creating a new alternative pathway for NTAP eligibility to reimburse for such services in the early years of their use.[8] **CMS could take a similar approach for reimbursement of these technologies under the OPPS by reimbursing services under the New-Technology Ambulatory Payment Classification (APC) until sufficient data is available to ensure that the resource use associated with AI services is appropriately packaged in the appropriate APC.**

Reimbursement under the PFS poses unique challenges. AI is unique from other technologies because it functions not only as infrastructure, but also as a direct clinical support tool embedded in patient care. Clinicians spend time reviewing, validating, and acting upon AI-generated outputs like chart notes, orders, and clinical decision support. CMS could consider adding Current Procedural Terminology (CPT) codes to address the work spent reviewing AI. This review process involves a level of professional judgment and medical decision-making. It can also be challenging to estimate the time spent reviewing an AI output, which might differ based on a clinician's comfort with the technology, how AI has adapted to clinician-specific workflow, and how the tool is embedded within hospital workflows. Thus, GNYHA recommends considering a non-time-based approach for determining work relative value units that better reflects the cognitive effort and individual-level variables affecting time spent reviewing AI outputs. Similar services such as review of radiologic imaging (e.g., CPT 76140: Consultation on X-ray examination made elsewhere, written report) are paid as services and non-time dependent. The calculation of practice expenses (PE) also presents a challenge. Traditionally, CMS has

[7] MedPAC. Report to the Congress: Medicare Payment Policy. March 2025. https://www.medpac.gov/wp-content/uploads/2025/03/Mar25_MedPAC_Report_To_Congress_SEC-1.pdf
[8] 42 CFR 412.87

classified such services as indirect practice expense, but as MedPAC points out in its June 2024 report to Congress, these expenses are not well accounted for in the physician practice expense information survey on which those costs are based.[9] GNYHA recommends that CMS instead classify these expenses as direct PE.

Research

To incentivize effective use of AI in clinical care, HHS should establish a dedicated Federal funding pathway to support real-world research on the implementation, evaluation, and scaling of AI-enabled clinical tools. While AI-related grants may be available through the National Institutes of Health (NIH) or other HHS agencies, there is no sustained, centralized program focused specifically on validating AI tools in clinical settings, assessing efficacy in clinical treatment, and evaluating safety and ROI. **A dedicated grant funding program within NIH or other Federal agencies could help accelerate research on AI**. The special AI funding program could be designed with a focus on specialties that show the most promise for breakthroughs in clinical care or show the most promise for high-value, cost-effective care. Over time, the list of specialties could be expanded as more promising studies emerge. Such a grant funding program should also prioritize participation from rural and safety net hospitals, through either dedicating a portion of funding specifically to these organizations, giving special consideration to their applications, or giving special consideration to a non-safety net organization that plans to fully integrate a rural or safety net hospital into its project.

As noted in the White House AI Action Plan, HHS should also prioritize the establishment of regulatory sandboxes or AI Centers of Excellence to allow researchers and established enterprises (including hospitals) to test AI tools in controlled, real-world environments while sharing data and results with peer organizations for educational purposes.[10]

3. ***For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?***

The use of non-medical device AI tools in clinical settings indeed raises novel legal issues. With respect to privacy and security, existing Federal laws and regulations, including HIPAA, the Health Information Technology for Economic and Clinical Health Act, and the Federal Trade Commission's breach notification rule provide general guidance but were not designed to address AI's unique challenges.

Key privacy and security issues include the appropriate limits on secondary data use for model training, performance improvement, and downstream commercialization of data, as well as ambiguity regarding what constitutes Business Associate status and related obligations in the AI space. Other issues include requirements for AI model transparency and auditability and how to address the expanded cybersecurity risk associated with enormous amounts of data being transferred and held for

[9] MedPAC. Chapter 4: Paying for software technologies in Medicare. June 2024. https://www.medpac.gov/wp-content/uploads/2024/06/Jun24_Ch4_MedPAC_Report_To_Congress_SEC.pdf
[10] The White House. America's AI Action Plan. The White House website. July 2025. Accessed January 22, 2026. https://www.whitehouse.gov/wp-content/uploads/2025/07/Americas-AI-Action-Plan.pdf

AI model training. This is especially important given the reliance on the relatively new technologies of application programming interface (API) integration and cloud-based processing environment.

Amidst all this change, hospitals, as frontline actors in the health care delivery system, remain accountable to patients and regulators, even when third-party developers and vendors design, host, or maintain these tools—increasingly by embedding tools in software in a way that is not transparent to hospital purchasers or end-users. While the market will drive a level of equilibrium, many hospitals—including most GNYHA members—lack the leverage to effectively negotiate with developers and vendors, especially the large companies producing most AI solutions. Given the complexity of AI model design and limits on what hospitals and end users are equipped to do to understand and explain how those models work, this exposure needs to be right-sized. **GNYHA recommends that HHS provide additional clarity regarding the application of HIPAA to AI model training, derivative data use, and data aggregation.** HHS could also direct the Health Sector Coordinating Council (HSCC) to develop standardized contractual and governance expectations for AI vendors, similar to the model contract developed for medical device manufacturers.[11] In addition, we believe that HHS and HSCC could support the development of AI–specific cybersecurity best practices to address emerging risks and strengthen sector-wide resilience, putting the focus on developers/vendors that design the systems and are in the best position to monitor outputs.

With respect to medical liability, developers and vendors understandably are using contractual indemnification and limitations on liability provisions to limit their exposure. However, while hospitals can and should establish their own internal governance and oversight structures, there are limits to how effective those structures can be in preventing or mitigating AI mistakes.

Establishment of applicable standards of care is generally the province of the states. Those standards will evolve as AI becomes more ubiquitous in the delivery of health care services. HHS, however, can play an influential role in managing AI medical liability by defining the relative roles of hospitals vis-à-vis developers and vendors through adapting existing standards to AI. **GNYHA urges HHS to address developer/vendor responsibilities for continuous monitoring throughout the lifecycle of AI tools and appropriate disclosure of training models and errors to hospitals and users**. Only with such disclosure will hospitals have the information they need to evaluate vendors' positions on indemnification and limitations on liability. Given the potential for AI tools to improve care and safety, it is crucial that adoption not be chilled by excessive litigation and legal exposure on those least able to bear it, especially in regions that already experience a high incidence of medical malpractice litigation and payouts, such as New York City.

[11] See HSCC Cybersecurity Working Group, Health Care Industry Cybersecurity Model-Contract Language for MedTech Cybersecurity-Version 2. Accessed February 18, 2026. https://healthsectorcouncil.org/wp-content/uploads/2025/11/MC2v2.pdf

4. ***For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?***

According to GNYHA members, certain technology vendors, such as EPIC, regularly provide reports to partner institutions on how effective the tool is and how it is being tested. This manner of continuous evaluation is needed to provide assurance to health care organizations that regular updates are being fully vetted before being rolled out to current customers. Schools of public health, biomedical research institutes, and other qualified entities should be provided with grants and cooperative agreements to develop and test metrics and evaluation methods that could become a best practice for private vendors and health care organizations. In soliciting applications for awards to study the best evaluation methods, HHS should consider various perspectives so that the most effective metrics for health care organizations, private vendors, and community members (which may not be the same) can be developed and implemented.

In seeding the development of evaluation methods, HHS should seek to promote any necessary disclosures from private vendors that will align their approach with the heavily regulated environment in which health care organizations work. This promotion of disclosure for private vendors can supplement evaluation measures taken at the hospital level, in addition to existing technical and cybersecurity review practices that are done at the hospital level.

5. ***How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?***

HHS can best support private sector activities to promote innovative and effective AI use in clinical care by building on the strong oversight and accreditation infrastructure that currently governs hospitals and health professionals. Hospitals, health systems, and clinicians operate within well-established regulatory and accreditation frameworks that address patient safety, quality improvement, and professional standards of care. As AI is integrated into clinical workflows, its use is already subject to oversight through these existing mechanisms. **GNYHA recommends that HHS not create additional reporting or regulatory requirements for hospitals and clinicians that are already subject to Medicare regulation and standards of accrediting bodies such as The Joint Commission (TJC), specialty boards, and state regulators.**

TJC standards and Medicare CoPs hold the governing body responsible for the safety and quality of care, treatment, and services. Hospitals have internal governance mechanisms that provide checks and balances to evaluate, implement, and monitor AI tools responsibly. HHS can help ensure that AI tools are deployed safely, effectively, and in a manner that strengthens those accountability systems in clinical care by leveraging the expertise and infrastructure within accreditation and existing regulations. By reinforcing and coordinating with the current oversight bodies, HHS can foster a regulatory environment that supports both innovation and high standards of clinical care.

Private vendors (non-health care facilities and non-clinical providers), however, may benefit from a national certification program focused on ensuring these private vendors can meet standards regarding safety, security, privacy, and quality. GNYHA safety net hospitals have reported difficulties evaluating promising AI products to know that they meet certain standards consistent with hospitals' already existing standards. **GNYHA recommends that HHS explore the creation of such a national certification program with the goal of giving assurances to health care entities that organizations meeting the certification standards have met a certain benchmark and can be further considered for partnership.**

6. ***Where have AI tools deployed in clinical care met or exceeded performance and cost expectations, and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?***

AI tools deployed in clinical care have met or exceeded performance and cost expectations in several targeted use cases, particularly where they address administrative burden, workflow inefficiencies, or high-volume diagnostic tasks. Ambient listening technologies that assist with clinical documentation have shown strong promise in improving note completeness and returning time to direct patient care—often without disrupting existing workflows. The use of the technologies may also contribute to reducing clinician burnout. Similarly, AI-enabled imaging tools in radiology and digital pathology are now widely used to support interpretation of CT scans, X-rays, ultrasounds, MRIs, and slides, helping to prioritize urgent findings, reduce turnaround times, and enhance diagnostic accuracy in specific use cases. AI-assisted patient monitoring systems, including video monitoring for fall prevention, patient positioning, and clinical deterioration detection, have improved patient safety while reducing reliance on resource-intensive one-to-one observation. AI-generated discharge summaries and other automated documentation tools also have demonstrated potential to improve care transitions and reduce administrative workload. However, some AI tools have fallen short when deployed without adequate clinical validation and workflow integration, particularly predictive analytics models that have not generalized well across patient populations or require burdensome data inputs.

In the future, AI tools with the greatest potential to improve health outcomes and reduce costs are those that deliver actionable insights at the point of care and are integrated into clinical decision-making. These include advanced early warning systems that more accurately predict sepsis, deterioration, or admissions; AI tools that synthesize longitudinal patient data to support complex chronic disease management; and real-time clinical decision support that incorporates imaging, laboratory, and genomic data. AI applications that identify gaps in care, support medication optimization, or personalize treatment pathways could meaningfully improve outcomes. In addition, AI tools that enhance operational efficiency—such as predictive staffing models, supply chain optimization, and capacity management—can help reduce system costs while preserving access. **To realize their full potential, however, AI technologies must demonstrate clear clinical benefits, interoperability with the EHR and other IT systems, and measurable ROI within real-world care environments.**

7. ***Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?***

Based on our members' experience, we understand the following hospital departments have a significant role in AI system adoption for clinical care: supply chain/procurement; IT; legal;

compliance; bioethics; clinical and nursing leadership; human resources; quality and safety; and medical education. Hospitals are either establishing governance committees dedicated to the oversight of AI use in their institutions or folding new responsibilities into existing governance structures, *e.g.,* technology procurement committees.

But whether this oversight activity is housed in a new or existing committee, it is more work for hospital administrators. Hospitals already suffer an enormous regulatory burden under Federal and state laws and regulations, each of which dictate their own infrastructure. Standards for AI governance and oversight are rapidly developing, and they will only add to this burden, potentially causing some over-stretched hospitals to delay larger-scale adoption as they tackle other priorities. In many hospitals, there is little bandwidth left to create new AI governance and oversight structures. For that reason, GNYHA strongly urges the Federal government to take the lead in developing standards for AI use in health care and to adapt and modify existing laws and regulations to that purpose rather than create entirely new frameworks. In adapting existing frameworks to AI use in health care, HHS should consider the staff time needed to comply with additional or different rules.

The AI regulatory framework is in flux, as HHS and the Administration have noted, with states increasingly exploring and enacting legislation as if they are operating in a regulatory vacuum. The uncertainty created by the Federal government's failure to take a clear, definitive stance on preemption represents another hurdle to adoption. **GNYHA supports the Administration's goal of developing a unified approach to AI regulation to the largest extent possible, leaving specific, narrowly defined unique interests to the states to address. If such a policy were enacted, a good deal of uncertainty would be removed from the market, allowing hospitals to more efficiently work with developers and vendors to bring AI tools into clinical care in more significant ways**.

8. ***Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.***

In recent years, Federal policy has meaningfully accelerated information sharing across the health care ecosystem. The information-blocking regulations have expanded baseline access to electronic health information, including via APIs; the Trusted Exchange Framework and Common Agreement (TEFCA) has established a nationwide governance framework for trusted exchange; and the Federal Health Technology Ecosystem initiative through CMS has encouraged broader data sharing, including with non-traditional health care entities, such as "big tech" and AI companies. Taken together, these Federal initiatives are increasing the volume, timeliness, and standardization of data available, including for clinical use. For AI developers, this expanding access to structured data creates new opportunities to train, validate, and deploy models.

However, expanded interoperability must be accompanied by clear guardrails. As additional entities gain access to hospital data via APIs, TEFCA, and other interoperability initiatives, **GNYHA recommends that HHS ensure disclosure on 1) which entities are authorized participants in interoperability frameworks, 2) the legal authority under which they can access health information, and 3) the scope of permissible uses, including downstream.** Given that CMS's Federal Health Technology Ecosystem initiative does not require a contract for participation (rather, organizations "pledge" to follow a framework), publicly available governance standards remain key.

GNYHA also recommends that HHS examine how state and local public health agencies could be better integrated into interoperability frameworks, particularly in relation to including their use of AI tools. Clear standards, defined use cases, and reciprocal exchange expectations could help ensure that public health participation delivers measurable value. For example, public health systems could participate in interoperability initiatives in a manner that allows for automation of the submission of reportable information using AI tools to analyze trends and provide actionable insights.

Finally, enhanced interoperability can accelerate AI development by enabling standardized access to structured clinical data. This includes laboratory results, medication histories, and imaging metadata encoded using nationally recognized standards, such as Fast Healthcare Interoperability Resources, the US Core Data for Interoperability, Logical Observation Identifiers Names and Codes, and Systematized Nomenclature of Medicine Clinical Terms. **HHS should promote consistent application of these standards and related data definitions across entities to strengthen transparency, benchmarking, and governance.**

9. ***What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?***

Patients and caregivers report persistent challenges in the health care system that they believe AI can help address. First, patients are seeking better access to care so that their conditions do not deteriorate and their needs are addressed in a timely manner. For example, AI-powered analytics and predictive tools can help identify at-risk patients most in need of preventive care. Second, patients are interested in more efficient health care systems, which can be improved by using AI-enabled scheduling tools to optimize appointments and the use of services (e.g., operating rooms). Third, patients and caregivers are seeking breakthroughs in research so more cures can be found for life-threatening diseases. Fourth, patients wish to have improved clinical outcomes, and they are hopeful that the promise of AI can contribute to breakthroughs in disease treatments.

At the same time, patients and caregivers express significant concerns about the adoption and use of AI in clinical care. First, patients are concerned about their data privacy and security, particularly among communities that are historically more distrustful of health systems. There is also widespread confusion about how AI tools are regulated and what consenting to use AI may entail, giving some patients a false sense of security about these tools. Clear education about how AI tools are developed, validated, and monitored, especially from private vendors, can promote trust. Second, patients are concerned about how health insurance organizations (payers) may use AI, especially that payers may use AI to deny medical claims from patients, doctors, and health care facilities. Patients are also concerned that payers may use AI to automate prior authorization processes without appropriate human oversight. HHS should work to ensure that payers do not use AI tools autonomously and that such tools are used only to support patient care rather than primarily to reduce costs or increase profit. Third, patients and caregivers are concerned about the use of generative AI in clinical decision-making without sufficient quality controls. However, hospitals are carefully monitoring the systems they use so that quality is improved and no unwanted outcomes occur. **Addressing these concerns (i.e., data privacy security, use of AI by payers to deny care, and quality controls when AI is used in clinical decision making)**

through strong oversight, clear guardrails, and explicit human involvement will be critical to ensure that AI enhances patient care.

10. ***Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?***

    a. ***Are there published findings about the impact of adopted AI tools and their use in clinical care?***

    b. ***How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?***

Existing research on AI in clinical care remains limited, particularly regarding whether generative AI models improve long-term patient outcomes. One recent study on the economics of AI in health care concluded that AI could result in significant cost savings when using AI tools in diagnosis and treatment.[12] While these and similar findings are encouraging, additional research should be done to assess the costs and benefits of using AI as a part of clinical care. Additional research, coupled with expanded opportunities to develop and test AI applications, is necessary to demonstrate the long-term benefits and costs associated with their use.

Most importantly, hospitals and health systems, clinicians, and patient perspectives should be consulted at each stage of developing AI research sponsored or led by the Federal government. Hospitals and clinicians can help ensure that research focuses on tools that are feasible to implement and integrate smoothly into clinical workflows and the existing IT infrastructure. Broader stakeholder engagement with health IT leaders, compliance experts, and community representatives can help identify operational and ethical considerations early in the design process. Including patient perspectives will also improve trust.

One emerging area where AI can significantly transform clinical care is through precision medicine, which uses information about an individual's genes, environment, and lifestyle to guide diagnosis and personalized treatment. While substantial advances have been made in this field, gaps remain between data analysis and practical clinical application. Large, complex data sets exist but are often underutilized because they are not easy to analyze. HHS should support research on AI tools that can synthesize these data, identify clinically meaningful patterns, and assist clinicians in developing tailored treatment plans. Increased Federal investment in precision medicine AI research would accelerate the integration of AI into specialty care, such as oncology and rare diseases, where individualized treatment strategies are particularly important. It would also help expand the number of health systems able to deliver this type of personalized medicine and increase accessibility to more patients regardless of their proximity to specialized providers or their ability to afford services.

HHS should also support expansion of AI in clinical trials, particularly to improve patient identification, recruitment, and retention. AI tools can more efficiently analyze EHR and claims

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[12] Narendra Khanna et al. *Economics of Artificial Intelligence in Healthcare: Diagnosis vs. Treatment*, National Library of Medicine. December 9, 2022. https://pmc.ncbi.nlm.nih.gov/articles/PMC9777836/

data to identify eligible participants and match them to appropriate trials. AI tools can additionally reduce administrative burdens that might delay site or patient enrollment. This would not only accelerate drug and device development but also expand access to innovative therapies for patients who might otherwise not be considered for participation. Such tools could help ensure that clinical research reflects the full range of patients seen in everyday practice by including clinical trial sites located in rural and underserved areas, minimizing bias, and broadening participation across different care settings. HHS must also develop strong regulatory standards for the use of AI in clinical trials to ensure that human subject research centers have proper patient protection.

Importantly, HHS should support research into health services: how AI can be used to improve health care delivery quality, safety, and efficiency. A key gap that research can address is how to integrate AI technologies to transform health care processes and improve decision making for patient care.

Lastly, HHS could make additional research grants and public-private partnerships available, in addition to the development of regulatory sandboxes that allow AI tools to be tested in controlled environments. These approaches would reduce barriers for hospitals and health systems of all sizes to participate in AI testing and research without regulatory noncompliance fears.

GNYHA appreciates the opportunity to provide these comments. Should you have any questions or wish to discuss any of these comments, please contact Tim Johnson at tjohnson@gnyha.org.

Sincerely,

Laura Alfredo	Erin DuPree	Tim Johnson
Executive Vice President, Legal, Regulatory, and Professional Affairs and General Counsel	Senior Vice President and Physician Executive, Quality and Clinical Initiatives	Senior Vice President, Center for GME, Health Workforce and Population Health